



Hospitals Cater to 'Transplant Tourists' as U.S. Patients Wait for Organs

International patients can bring a hospital as much as \$2 million for a transplant. In recent years, they have typically gotten organs faster than U.S. patients.



Alyssa Schukar for The New York Times

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By **Brian M. Rosenthal** and **Mark Hansen**

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Heart transplant patients in the United States typically spend months waiting for a donated organ. But Kayoko Hira was not a typical patient.

Mrs. Hira, the wife of a hotel magnate in Japan, flew to the United States in September 2021, went to the University of Chicago Medical Center and, within days, got a new heart from an American teenager who had died.

Soon after, The New York Times found, a charity run by her husband made a donation to a nonprofit group led by the heart surgeon's wife. It was the only time the charity has ever given money to an American institution, according to its website.

More than 100,000 people in the United States are in need of a transplant, and each year thousands die waiting. But despite the shortage of organs, some American hospitals are aggressively courting international transplant patients, a New York Times investigation found.

They have advertised abroad, promoting short wait times and concierge services, particularly to patients in the Middle East, where about two-thirds of overseas transplant recipients are from. Several hospitals have signed contracts with foreign governments, setting prices for different organ transplants.

An international transplant patient can bring in as much as \$2 million — far more than a U.S. patient paying through private insurance or a public program like Medicare.

In the past dozen years, more than 1,400 patients from abroad received a transplant in the United States after traveling specifically for the procedure. That was a small fraction of all U.S. transplants, and most transplant centers did not operate on international patients at all.

But The Times found that a handful of hospitals are increasingly catering to overseas patients, who make up an ever-larger share of their organ recipients: 11 percent for hearts and lungs at the University of Chicago; 20 percent for lungs at Montefiore Medical Center in the Bronx; 16 percent for lungs at UC San Diego Health; 10 percent for intestines at MedStar Georgetown University Hospital in Washington; and 8 percent for livers at Memorial Hermann-Texas Medical Center in Houston.

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Montefiore Medical Center in the Bronx is among a small number of U.S. hospitals increasingly catering to international transplant patients. Dave Sanders for The New York Times

In many countries, this would be illegal. World leaders agreed in 2008 to fight so-called transplant tourism, and most nations do not provide organs to overseas patients. Yet the United States has long allowed it. The policy has drawn criticism in the past, such as when organs went to [Saudi royals](#) and a [Japanese crime boss](#).

The Times analyzed every transplant performed in the United States since 2013. Overall, it found that patients who traveled from other countries received transplants faster than patients from America and were less likely to die waiting for an organ.

Dr. Mark Fox, a former chair of the transplant system's ethics committee, said the findings were troubling, especially because overseas patients do not contribute to America's pool of donated organs. "The unfortunate reality is that we don't have enough organs," he said. "When people jet in, get an organ and jet home, it's a problem. It's not fair."

In response to requests for comment, several hospitals emphasized that giving transplants to international patients is legal, and said those patients do not get preferential treatment. Some described their work as humanitarian.

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The transplant system used to effectively cap the number of transplants for overseas patients, focusing mostly on rare cases of particular need. Then, The Times found, things changed.

Reaching New Patients, and Profits

Irene Gebrael was an Arabic interpreter at Montefiore when, she said, she had an idea: If the hospital recruited international patients, it could serve a broader population — and make a lot of money.

"The hospital is always happy about the international rates," Ms. Gebrael said in an interview. "Trust me."

In 2013, she said, she persuaded hospital executives to start an international office. It was good timing for the transplant program.

For decades, hospitals that gave more than 5 percent of any organ type to "noncitizen nonresidents" could be audited and possibly penalized. But leaders of the transplant system ended that in 2012, concerned that it could lead to discrimination against undocumented immigrants, a group that commonly donated organs. (The Times analysis did not include patients who traveled to the United States for reasons other than transplants, such as immigrants.)

Since then, Montefiore, a modest transplant program in a low-income section of New York City, has increasingly treated international patients, which Ms. Gebrael said generated tens of millions of dollars in revenue. From 2020 to 2024, it provided organs to 49 such patients, the third most of any U.S. hospital.

"You make the hospital gain money, and the patients are happy, the embassies are satisfied," said Ms. Gebrael, who left Montefiore in March and started her own business coordinating health care for overseas patients.

In a statement, Montefiore said it "strictly adheres" to all transplant system policies.

Since the restriction on international patients was lifted, only about 25 of America's roughly 250 transplant centers have given more than one organ per year, on average, to overseas patients, The Times found.

At the hospitals that have provided the most transplants in recent years, nearly all of the international recipients came from Kuwait, Saudi Arabia, Israel, the United Arab Emirates and Qatar.

U.S. hospitals have advertised in Arabic-language magazines, hosted conferences in the Middle East and hired consultants there. "We are a world leader in transplant," said a University of Chicago video posted with Arabic subtitles. It promised services including airport pickups and lodging assistance.

The University of Chicago performed transplants for 61 international patients from 2020 through 2024 — more than any other center.



The University of Chicago Medical Center has courted international patients through advertisements and made deals with foreign governments. Jamie Kelter Davis for The New York Times

The hospital declined to answer questions about treating overseas patients. It issued a statement saying it had "significantly improved access to organ transplantation in recent years, driven by our commitment to the health and well-being of the diverse and

underserved populations of Chicago's South Side and the broader region.”

Hospitals have competed to win over Middle Eastern governments, which often pay to send citizens abroad for care. Under federal rules, hospitals are supposed to consider each transplant patient individually and cannot strike broader agreements with governments. But at least three hospitals — the University of Chicago, Montefiore and Memorial Hermann — have signed such deals, according to former employees.

Those hospitals did not respond to questions about the contracts. A spokeswoman for UC San Diego said it had arrangements with Israeli health insurers. Data shows that it gave organs to 37 Israelis from 2020 to 2024, more than all other centers combined.

UC San Diego said international patients are referred there when they are especially high-risk or cannot get procedures in their home countries.

Dr. Mustafa Al-Mousawi, president of the Kuwait Transplant Society, said his government had agreements with multiple American hospitals. But because the government had to pay more than \$1 million for each transplant, it did not send over everyone in need. The wealthy, he said, tend to have better access to the officials deciding who gets to go.

“This is unfortunate,” he said, “because there are many poor people in Kuwait who need transplants, too.”

‘He Is International 🌍’

The transplant system is designed to prioritize fairness: All potential recipients must join a national registry, and organs from deceased donors are distributed to patients based on factors such as how sick they are and how long they have been waiting.

But despite the safeguards, international patients have gained advantages.

In interviews, 25 current and former employees of eight hospitals said they had seen overseas patients receive preferential treatment.

To join the registry, patients must be certified by a hospital that they can survive a transplant and care for a new organ. But non-American patients are often fast-tracked, said workers who have evaluated potential organ recipients at the University of Chicago, Montefiore, UC San Diego and Georgetown. Some said they had been overruled by superiors after rejecting an international patient who smoked, had a high body mass index or otherwise did not meet criteria.

In text messages obtained by The Times, workers at one of those hospitals discussed how an overseas patient would inevitably make it onto the registry: “We will do him,” one said, “he is international 🌍.”

The hospitals said they fairly considered all patients to ensure they qualified for a transplant. “Our medical center doesn’t expedite listing evaluations for any patient,” the University of Chicago said.

When organs are retrieved from deceased donors in the United States, they are offered to potential recipients in priority order, as determined by algorithms.

But hospitals can request an “exception” to raise a patient’s priority level if they believe the algorithm does not fully capture the state of the person’s health. These requests go to committees of doctors where they are usually approved without much scrutiny, according to interviews with three physicians who do reviews.

In recent years, international patients who received lung or liver transplants were more likely than Americans to have gotten exceptions. For lung transplants, the data shows, 23 percent of overseas recipients got an exception; less than 5 percent of the Americans did. The rates were about even for hearts, the only other organ that allows for exceptions.

Kyella Fonseca, who worked as a financial coordinator for transplants at Montefiore from 2020 to 2022, said she had seen preference given to international patients, who she said generally paid more. She said colleagues sometimes altered or omitted health information in their medical records to help them get organs more quickly.

“We had patients who we’d been working with, who had been waiting their turn, and then someone from Kuwait would come and jump the line,” she said.

In its statement, Montefiore said, “The suggestion that we would compromise the health and well-being of any U.S. patient is unequivocally false.”

A Transplant in 3 Days

When Mrs. Hira arrived at the University of Chicago, it was clear that she was a V.I.P., according to two people involved in her care who requested anonymity because they were not authorized to discuss patients.



Kayoko Hira, the wife of a hotel magnate in Japan, got a heart transplant at the University of Chicago three days after going on the waiting list. Typically, patients in the United States wait months.

Mrs. Hira, then 55, was married to Ryuko Hira, the owner of the HMI Hotel Group, one of Japan’s largest hotel chains. She and her husband also had something in common with the hospital’s top heart surgeon, Dr. Valluvan Jeevanandam. They were all followers of the Indian spiritual figure Sathya Sai Baba.

She had faced long odds of getting a new heart in Japan, where only about 10 percent of residents are registered donors and the wait for a transplant can stretch for years.

On Sept. 30, 2021, the University of Chicago added Mrs. Hira to the registry and obtained an exception that increased her priority level, records show.

The same day, a 19-year-old woman died in the Midwest. The transplant system's algorithm identified more than 400 potential matches for her heart — including Mrs. Hira.

Patients often wait months to gain priority and for the right organ to become available. But with the exception, Mrs. Hira was No. 7 on the list. Doctors for the patients above her determined the organ wasn't right for them, and then the University of Chicago accepted. Dr. Jeevanandam performed the transplant on Oct. 3.

Mrs. Hira got her transplant faster than 91 percent of heart recipients that year, data shows.



Dr. Valluvan Jeevanandam, Mrs. Hira's surgeon, spoke at a transplant conference in Kuwait in November. Christophe Viseur for The New York Times

She was a “self-pay” patient, meaning she didn’t use insurance, according to records reviewed by The Times. Such international patients often pay the full billed amount, which averages about \$1.9 million for a heart transplant, according to [recent research](#).

The month after the transplant, the Sai Hira India Foundation, a charity founded by Mr. Hira, donated money to the Sai Spiritual Foundation, a Chicago nonprofit that promotes the teachings of Sai Baba and performs community service. Tax filings show that the Chicago nonprofit's principal officer was Sheela Jeevanandam, the surgeon's wife. Public records do not disclose the amount of the donation; that year, the group reported raising about \$1 million more than any year before or since.

The University of Chicago declined to discuss Mrs. Hira's case, citing patient privacy laws. A representative for Mr. Hira declined to comment, and Dr. Jeevanandam did not respond to requests for comment.



Mrs. Hira's husband, Ryuko Hira. After the transplant, a charity he runs made a donation to a nonprofit group led by the heart surgeon's wife.
Sipra Das/The India Today Group/Getty Images

Dr. Gabriel Danovitch, a longtime leader of the U.S. transplant system and a co-author of the 2012 policy change on international patients, said that transplant tourism was still very uncommon in America. But he said he was greatly disturbed to hear that an overseas patient had so quickly received an organ and made a donation. "Truthfully, it makes me sick," he said.

About three weeks after her transplant, Mrs. Hira had recovered enough to leave the hospital. She told her caregivers there that before returning to Japan, she wanted to give them something to remember her. They each received a jacket embroidered with their name, the date of Mrs. Hira's transplant and the lotus flower symbol of Sai Baba.

Reporting was contributed by Rachelle Bonja, Javier C. Hernández, Hiroko Tabuchi, Hisako Ueno and Steven Rich. Julie Tate and Susan C. Beachy contributed research.

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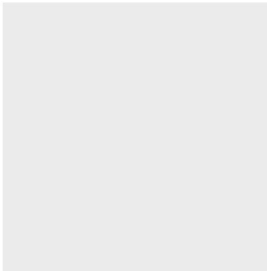
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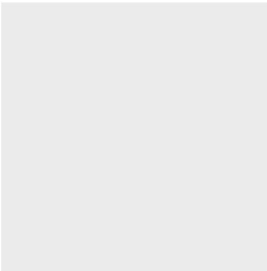
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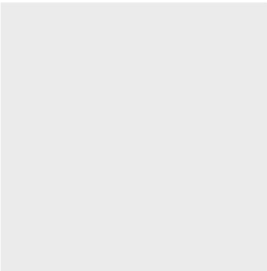
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